

41. PRACTICE 2 THE COLIC FRAMEWORK

DURATION : 03:23

STUDY SHEET

COURSE SUMMARY

Presentation of post-caesarean body revitalisation techniques, focusing on the colic framework and the peritoneum. Palpatory examination of the abdomen including tissue and thermal response to restore fascial homogeneity. Analysis of the terminal fulcrum, tissue releases and dynamic adjustment required after pelvic interventions (e.g. hysterectomy). Clinical methodology to promote harmonious tissue reintegration.

PRACTICE 2: THE COLIC FRAMEWORK

A **caesarean section** severs the body between top and bottom, requiring **redynamisation**. The uterus represents a concentration of **posterior parietal peritoneum**. During development within the peritoneum, there is a concentration and power that recentres, forming a **terminal fulcrum**. This fulcrum is both embryonic and final, and it is crucial to bring them back into consciousness.

During palpation, it is essential to take the entire abdomen in the hands, keeping the hands well relaxed. Breathing too high can indicate a request for **discretion**. The **heat** felt is an exchange between the practitioner and the patient, where the tissue begins to provide information. If the tissue goes into defence, the lower area begins to relax, while the upper part still lacks homogeneity. The goal is to regain a homogeneous state.

When the heat increases, all lines of force seem to orient towards the uterine area, as if the tissue were reintegrating its **nest**. This helps the patient return to their **inner home**, similar to someone attached to their land. At this stage, the practitioner can broaden their contact to the entire body, initiating a **therapeutic process**. Integration towards the uterine area strengthens, and greater homogeneity is created between the hands.

In the case of a **hysterectomy**, the body's dynamics change, and the patient may develop a **buffalo hump**. This requires rebalancing in several areas. The work focuses on tissue points, allowing for tissue release and synchronised development. Support points are essential, particularly to force the **diaphragm** to work differently, seeking other spaces and openings.

The practitioner can then address specific areas such as the hepatic and sigmoid, allowing for complete relaxation.

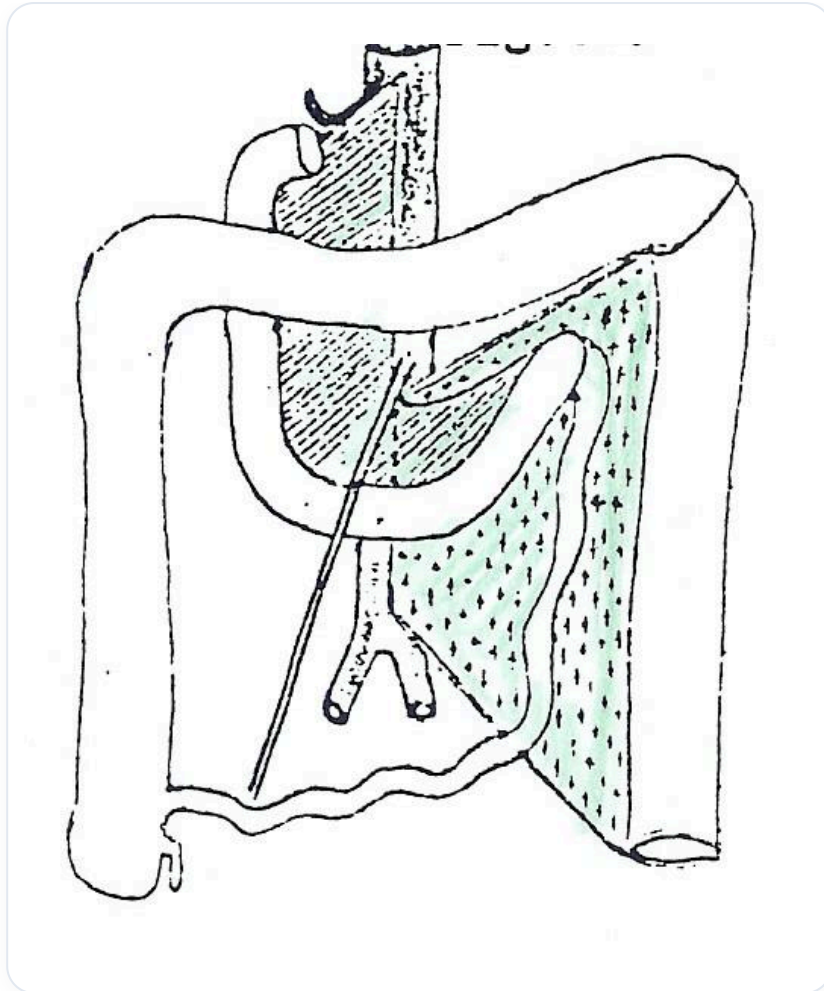


FIG. — Colon anatomy